

CORY SHEPHERD

Administration Officer, Switchboard Services, Logan Hospital | Employee no. 388372
coryshepherd1@hotmail.com | 0417 400 227

Supporting statement – two applications for sick leave without pay

Recoding of two periods presently recorded as leave without pay, at the request of Metro South Payroll · 11 September 2026

1. What is applied for

Two applications for **sick leave without pay**, for the two periods identified by Metro South Payroll on 7 and 8 September 2026. The dates are Payroll's own and are not varied.

Application	Period	Days (inclusive)
1	21 June 2024 – 20 September 2024	92
2	13 December 2024 – 23 February 2025	73

The periods are presently recorded as leave without pay. Nothing else is sought by these applications, and no other period is affected.

2. Payroll has already identified the correction and the step required

On 7 September 2026 at 1:28 pm Metro South Payroll wrote to the line manager:

“Could I please confirm with you whether the following periods were intended to be recorded as Sick No Pay rather than Leave Without Pay? 21.06.2024 - 20.09.2024 / 13.12.2024 - 23.02.2025. This will affect the employee's LSL date... we would require a leave form to be submitted for both periods. Could you please confirm?”

On 8 September 2026 at 2:24 pm Ms Grace Strachan, Senior Payroll Officer, wrote to me:

*“We believe that the periods 21/06/2024 - 20/09/2024 and 13/12/2024 - 23/02/2025 **should have been booked as Sick No Pay (SNP) rather than Leave Without Pay (LWOP)**... If these periods need to be changed to Sick No Pay, new leave forms will be required for both periods. Once the leave forms are received and the leave is updated accordingly, **we can recognise the first three months of each period towards your LSL date.**”*

These applications are made so that the leave forms Payroll requires are on foot. I am not asking anyone to accept a new position; the position is Payroll's own.

3. Application 1 – 21 June 2024 to 20 September 2024

The absence was medical from the outset and is documented throughout:

- My leave record shows **sick leave on 7, 8 and 9 June 2024**, including sick leave without pay on 9 June, and leave without pay from 10 June 2024.
- A **medical certificate was issued on 28 June 2024**.
- The **work capacity certificate of Dr Peter Hawes dated 1 July 2024** records the date of injury as **18 June 2024** and that I was first seen for that injury on 1 July 2024.
- That certificate certifies **no functional capacity for any type of work over the period 1 July to 6 October 2024** – a period which contains the whole of this application from 1 July onward.
- Further work capacity certificates were issued on **7 August 2024 (Dr Ki Pang)** and on **11 August and 8 September 2024 (Dr Hawes)**.

- Review Decision 69983 records that the certificate of 1 July 2024 indicated “there was no pre-existing factor or condition”, and that this “was maintained in all later work capacity certificates”.
- My application for workers’ compensation was **lodged on 1 July 2024** and rejected on 13 September 2024. That rejection is under appeal and is not a matter for this application.

4. Application 2 – 13 December 2024 to 23 February 2025

The absence was a continuation of the same medical condition:

- **On 24 October 2024** my treating psychiatrist, Dr Ravikumar Bangalore Krishnaiah, diagnosed **Major Depressive Disorder with anxiety state** at the first consultation, and confirmed that diagnosis in writing the same day.
- **On 13 February 2025** – within this period – Dr Krishnaiah provided a report recording the continuing diagnosis, its severity and functional effect, and the treatment then in place.
- The period ends on **23 February 2025**, being the day before a return to work was confirmed for **7:00 am on 24 February 2025**.

If a work capacity certificate specific to this period is required, please let me know and I will obtain it from the treating practice. I would rather be told what is missing than have the application held.

5. The effect, on Payroll’s own figures

Payroll’s calculation of 4 September 2026 records my commencement with Queensland Health as **25 March 2019**, and that eligibility would ordinarily arise on **25 March 2026**. Payroll has identified **189 days presently excluded**, being 24 days (18 May – 10 June 2022), 92 days (21 June – 20 September 2024) and 73 days (13 December 2024 – 23 February 2025), which moves the date to **29 September 2026**.

On Payroll’s stated approach – that on recoding “the first three months of each period” are recognised – each of these two periods falls **entirely** within its own first three months: three months from 21 June 2024 ends 21 September 2024, and the period closes on 20 September; three months from 13 December 2024 ends 13 March 2025, and the period closes on 23 February. On that arithmetic **165 of the 189 days return**, leaving only the 24 days of 2022, and the resulting date falls in **April 2026**. The calculation is of course Payroll’s to make.

6. Reservation

I have received no wages for work since 13 July 2026. Since 3 July 2026 I have been certified fit for duties with restrictions on the Employee Capability Checklist completed by Metro South Health, I have remained available, and I have not been provided with work. What I have been paid since 13 July has been drawn from my own leave accruals and not paid as wages.

Any leave applied for, taken or paid in that period is applied for and taken under protest and without prejudice to my claim for wages for the period I have not been provided work, and I reserve the right to seek reinstatement of the leave taken. These two applications relate to 2024 and 2025 and are made solely to give effect to the recoding Payroll has identified; they are not an acceptance of any position about 2026.

Signed



Cory Lea Shepherd

Administration Officer, Switchboard Services, Logan Hospital
Employee no. 388372

Date: 11 September 2026

Attached: work capacity certificate of Dr Peter Hawes signed 8 September 2024 (2 pages). Further certificates of Dr Ki Pang dated 7 August 2024 and Dr Peter Hawes dated 11 August 2024 can be provided on request. The certificate of Dr Peter Hawes dated 1 July 2024 is held by the Workers' Compensation Regulator at item 7 of its list of documents.

Part A - Patient details

Name: Cory Sheppard Date of Birth: 11/01/1981
 Mobile number: 0833408827 Claim number: 8054943001 ☐ New claim ☐ Claim is report only
 Occupation (if known): Admin work Patient's employer: Qld Health

Part B - Injury details

Date of accident: 11/08/2004 Patients stated date of injury: 08/09/2004 Patient was first seen at this practice/hospital for this injury/illness on: 01/07/2004
 Date of assessment: 11/08/2004

The patient is/was suffering from: Left elbow related symptoms
 If reported only, tick "Threatened diagnosis" ☐ Professional diagnosis: acute stress
 Patient's chief mechanism of injury: terminal at work Is this consistent with your clinical findings? ☐ Yes ☐ Unclear
 ongoing breaking of workplace rules by patient, violating him

Describe symptoms in detail
 Presenting symptoms in detail
 appropriate (not previously reported)

Part C - Treatment plan

Patient requires treatment for: 08/09/2004 to 08/10/2004 to be reviewed again on: 08/10/2004 ☐ No further review
 Treatment: ongoing therapy How this treatment
 I have provided treatment for my injury/s with work, travel or cognitive function: ☐ No ☐ Yes
 Medication: ibuprofen ☐ Other: None Name of discipline: Physiotherapist Details (specify):

Part D - Capacity for work

☐ The condition will not prevent a return to work ☐ If suitable duties available, can return to some form of work ☐ No functional capacity for any type of work until
 Pre-injury duties: not complete part B, Q4 to Part F. from

Complete the next section if you certified no functional capacity for any type of work

If no functional capacity, state why? then capacity for more than 1 day. The insurer may contact you to obtain more information.
 Estimated time to return to some form of work duties: unknown Estimated time to return to full duties: unknown

Part E - Functional ability

Efforts to improve medical/psychosocial symptoms, functional capacity to complete: No change since last certificate
 Certification should be based on what DW to do, not available duties. Consider what the patient can do either at work or home.

Function/task (patient's usual) Is functional ability affected by injury/illness? What patient can do if "yes" is selected?
 Functional ability: No/Yes Note any restrictions (if relevant)
 Lower limb: No
 Upper limb: No
 Hand function: No
 Spinal function: No
 Cognitive/psychosocial functioning: No
 Driving a car: No
 Operating machinery/vehicle: No
 Manual tasks: No
 Other: No

Part F - Rehabilitation at work - return to work plan

Options for emergency medical/psychosocial symptoms, functional capacity to complete: No change since last certificate
 What workplace modifications are required to facilitate return to work? a. work site assessment, psychosocial support

Other considerations or factors that may affect recovery: the insurer can arrange appropriate support

☐ I require a suitable duties program to be provided to me for approval
 I have discussed injury requirements and return to work with the patient and: ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (for use with hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.
 Name: Peter Haines Email: phaines@nursepractitioner.com.au
 Practice/hospital: Our Medical Associates Phone: 0734478000 Date: 08/08/2004
 Postal Address: 200 Glen Avenue, Adelaide
 QLD 4214 Signature: [Signature]

Further information: www.worksafe.qld.gov.au/medicalsupport

At completion (Medical/dental/nurse practitioner, patient, employer) call 081 122

Under the Workforce Compensation and Rehabilitation Act 2005 (the Act), the employer/insurer must be satisfied to return the information on this form to provide by the worker's capacity to perform work. The information provided on this form may be used by the insurer/employer/insurer to provide a suitable duties program to the worker. The worker's capacity to perform work is assessed on the basis of the information provided in this form. The worker's capacity to perform work is assessed on the basis of the information provided in this form. The worker's capacity to perform work is assessed on the basis of the information provided in this form.



